

MINNESOTA HEALTH CARE PROGRAMS (MHCP)

Fee-for-Service (FFS) only or FFS and Managed Care Organization In-Network Provider Agreement

Use this form if you or your entity provide services to MHCP fee-for-service (FFS) members OR you are a provider providing services to MHCP FFS members **AND** you are an in-network provider with a Managed Care Organization (MCO) to provide services to MHCP managed care members.

As a participating provider in Minnesota Health Care Programs (MHCP) administered by the Minnesota Department of Human Services (DHS), the provider agrees to:

1. Furnish DHS, the Secretary of the U.S. Department of Health and Human Services, or the Minnesota Medicaid Fraud Control Unit with such information as it may request regarding payments claimed for services provided under these programs.
2. Comply with all federal and state statutes and rules relating to the delivery of services to individuals and to the submission of claims for such services.
3. Provide to DHS or contracted managed care organization (MCO) its national provider identifier (NPI) and include its NPI on all claims, if the provider is eligible for an NPI.
4. Comply with all provisions of [Minnesota Statutes, 62J.536](#), which requires electronic transmission of claims, eligibility and other transactions, using DHS' secure, HIPAA-compliant, automated transaction tool MN-ITS, or the contracted MCO's tool, or a HIPAA-compliant tool approved by the MCO.
5. Accept as payment in full, amounts paid according to schedules established by DHS, except where DHS authorizes payment by the member.
6. Enroll in electronic funds transfer (EFT) if the provider is a pay-to provider and if DHS requests that.
7. Ensure, when required by law, that a health service program administered by DHS or by the contracted MCO is the payer of last resort by determining the legal and financial liabilities of third parties to pay for covered services, and pursuing such third-party payments.
8. Assume full responsibility for the accuracy of claims submitted to DHS or by the contracted MCO according to the certification requirements of the [Code of Federal Regulations, title 42, section 455.18](#) and [Minnesota Statutes, 256B.27](#), subdivision 2.
9. Submit claims at no more than the provider's usual and customary fee to the general public and only after the medical care or service has been provided, according to [Minnesota Rules, 9505.0450](#), subpart 1.
10. Except for claims for services under a waiver program, submit claims only for services, supplies, and equipment that are medically necessary as defined at [Minnesota Rules, 9505.0175](#), subpart 25, and that meet professionally recognized standards of health care that the provider knows or has reason to know are properly reimbursable under federal and state statutes and rules.
11. Make full disclosure of ownership and control information as required by the [Code of Federal Regulations, title 42, sections 455.100 - 455.106](#), and upon request, full disclosure of business transactions, as is required by the [Code of Federal Regulations, title 42, section 455.105](#).

Electronic initials accepted.

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12. Make full disclosure of persons convicted of program crimes as required by the [Code of Federal Regulations, title 42, section 455.106](#).
13. Ensure that the provider, all of its owners, managers, employees and contractors are not excluded from participation in Medicare, Medicaid or other federal health care programs, by searching the [Office of Inspector General List of Excluded Individuals and Entities \(LEIE\)](#). The provider must conduct this search at the time of enrollment, before hiring new employees or entering into a contract with a contractor, and monthly to see changes since the last search. The provider must immediately report any exclusion information discovered to DHS and any contracted MCOs.
14. Verify member eligibility before rendering services.
15. Comply with all federal statutes, implementing regulations and guidance prohibiting discrimination on the basis of race, color, national origin, sex, age, religion and disability in any program or activity receiving federal financial assistance from the Department of Health and Human Services.
16. Provide member services of the same scope and quality as would be provided to the general public, within MHCP guidelines, according to [section 1902\(a\)\(10\)\(B\) – \(E\) of the Social Security Act](#).
17. Comply with the provisions of any fully executed addendum required by DHS, which is incorporated with the Provider Agreement (that is, the addendum becomes part of the original Provider Agreement).
18. Ensure that its employees and contractors comply with all MHCP requirements and any contracted MCO requirements, including any requirements added post-enrollment.
19. Comply with the advance directive requirements if the provider is a hospital, nursing facility, a provider of home health care, personal care assistance services, or hospice, as required by the [Code of Federal Regulations, title 42, sections 489.102 and 417.436](#).
20. Maintain records that fully disclose the extent of services provided to MHCP members for a period of five years after the initial date of billing DHS or a contracted MCO, according to [Minnesota Rules, 9505.2160 – 9505.2245](#), or for the duration of contested case proceedings, whichever is longer.
21. Ensure proper handling and safeguarding by the provider's employees, contractors, and authorized agents of protected information collected, created, used, maintained, or disclosed on behalf of DHS. For the purposes of this agreement, "protected information" means data subject to any of the laws described in 21.A. This responsibility includes:
 - A. Ensuring that employees and agents of the provider comply with and are properly trained about:
 - (1) The Minnesota Government Data Practices Act (MGDPA), [Minnesota Statutes, Chapter 13](#), in particular [13.46 Welfare Data](#);
 - (2) The Minnesota Medical Records Act, [Minnesota Statutes, 144.291 – 144.298](#);
 - (3) The federal Health Insurance Portability and Accountability Act (HIPAA) including, but not limited to, the requirements of the Privacy Rule and Security Regulations, the [Code of Federal Regulations, title 45, sections 160 and 164](#);
 - (4) Federal law and regulations that govern the use and disclosure of substance abuse treatment records, [United States Code, title 42, 290dd-2](#) and the [Code of Federal Regulations, title 42, sections 2.1 – 2.67](#); and
 - (5) Any other applicable state and federal statutes, rules, and regulations affecting the collection, storage, use and dissemination of private or confidential information.

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B. Ensuring, consistent with the laws stated in 21.A, that the provider's employees, contractors, and authorized agents:

- (1) Do not use or further disclose protected information created, collected, received, stored, used, maintained or disseminated in the course or performance of this agreement other than as necessary to perform their obligations under this agreement, or as required by law, either during the period of this agreement or after (respectively, the [Code of Federal Regulations, title 45, sections 164.502\(b\) and 164.514\(d\)](#), and [Minnesota Statutes, 13.05](#), subdivision 3).
 - (2) Use appropriate administrative, physical, and technical safeguards to prevent use or disclosure of the protected information other than as provided for by this agreement and to ensure the confidentiality, integrity, and availability of any protected health information that it creates, receives, maintains, or transmits on behalf of DHS.
 - (3) Do not transmit protected health information (PHI) over the internet or any other unsecure or open communication channel unless such information is encrypted or otherwise safeguarded using procedures no less stringent than those described in the [Code of Federal Regulations, title 45, section 164.312](#). If the provider stores or maintains PHI in encrypted form, the provider shall, at DHS' request, promptly provide DHS with the key or keys to decrypt such information. The provider shall not forward previously encrypted data to any other party, unless otherwise required by this agreement.
 - (4) Mitigate, to the extent practicable, any harmful effects known to the provider of a use, disclosure, or breach of security with respect to protected information by the provider in violation of this agreement.
 - (5) Make the required notifications upon discovery of a breach, as defined in the [Code of Federal Regulations, title 45, section 164.402](#), of unsecured PHI to DHS, to each individual whose unsecured PHI has been breached, and, when the breach involves the unsecured PHI of more than 500 people, to the media of a state or jurisdiction. Refer to [Code of Federal Regulations, title 45, sections 164.400 – 164.414](#).
22. Accept and be bound by the terms and conditions of DHS' [Electronic Data Interchange \(EDI\) Trading Partner Addendum to Provider Agreement](#) when billing electronically. The provider acknowledges that any organization or individual that submits claims on its behalf will abide by the EDI Trading Partner Agreement as an agent of the provider. The provider authorizes the agent to bind the provider to the terms of the EDI Trading Partner Agreement. The provider will give each EDI trading partner an individual login ID and password.
23. For provider entities receiving or making Medicaid payments totaling at least \$5 million annually, establish written policies and procedures for the education of all employees, contractors and agents, that includes information about the False Claims Act and other provisions named in [section 1902\(a\)\(68\)\(A\) of the Social Security Act](#).
24. Determine the applicability to the provider of any other state or federal laws and ensure compliance with those laws.
25. Cooperate with DHS or contracted MCO audit or review procedures.
26. Execute any required assurance statements and provide certification or licensure information if required by DHS for a particular provider type. The provider also agrees to notify DHS of any changes to its certification or licensure status.
27. Comply with [Minnesota Statutes, 256B.0644](#) as a requirement of participation in other state health care programs. The provider agrees to provide active caseload data upon DHS' request and at least 10 days before limiting acceptance of new MHCP members.

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28. Refund any overpayments made to the provider by DHS or the MCO, including those resulting from payments made by Medicare, third party payers, billing errors, fraudulent billing, and from increased interim payments made pursuant to DHS' plan for continuity of operations during times of pandemic and crisis.
29. Notify DHS no later than 30 days before the effective date of a sale, merger, or transfer of an enrolled entity, according to [Minnesota Rules, 9505.0195](#), subpart 8. Failure to notify DHS may result in the sale or transfer not becoming effective with DHS for any purpose, including claims processing, payment of claims and claims adjustments. The provider also agrees to notify DHS whether it intends to transfer its NPI or its Federal Employer Identification Number (FEIN) to the new owner and to complete any documentation or addenda DHS requires, including a [Provider Entity Sale or Transfer Addendum \(DHS-5550\) \(PDF\)](#). The provider acknowledges that upon sale, merger or transfer of the enrolled entity, DHS will recognize the effective date of the sale or transfer as the date from which all claims payments or adjustments will be assigned to the new owner, without regard to date of service, date of submission to DHS, or adjudication date, including those resulting from a later audit or reprocessed claims. Any intent on the part of the provider or purchaser to the contrary must be addressed in the purchase agreement and transfer documents and is the responsibility of the provider and purchaser to enforce. DHS retains the right to pursue monetary recovery, or civil or criminal actions against the seller or transferor. Nothing in this agreement negates the obligation of the new owner to contact DHS by the effective date of sale, merger or transfer.
30. Accept that this agreement may be immediately terminated for either of the following:
 - A. At the discretion of DHS if it determines that the provider has violated a material term of the agreement, including but not limited to:
 - (1) Noncompliance by the provider with the HIPAA Privacy Rule and Security Standards. If termination is not feasible, DHS shall report the breach to the secretary of Department of Health and Human Services.
 - (2) Failure of the provider to sign a new agreement within 30 days of a request from DHS, in accordance with [Minnesota Rules, 9505.0195](#), subpart 5.
 - B. Upon sale or transfer of the enrolled provider.
31. Ensure that upon termination of this agreement, the provider shall continue to:
 - A. Extend all of the protections of this agreement to all of the protected information DHS provides to the provider, or created or received by the provider on behalf of DHS, that the provider still maintains in any form, including information that is in the hands of the contractors and agents of the provider, and limit its further use and disclosure.
 - B. Maintain all other records of claims submitted for a minimum of five years, consistent with paragraph 20 of this agreement.
32. Accept that any ambiguity in this agreement will be resolved to permit DHS to comply with HIPAA, MGDPA, and other applicable state and federal statutes, rules, and regulations affecting the collection, storage, use and dissemination of private or confidential information and other state and federal laws and regulations.

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Signature requirements for this Provider Agreement

Individual providers

- Type your name, NPI or Unique Minnesota Provider Identifier (UMPI), and the provider type on the first page of this agreement. This information will automatically populate to pages 2 and 3 after you enter it.
- Initial each page of this Provider Agreement.
- Write the name and title of the person signing and sign the last page of this agreement.

Organizational providers

- Type your organization name, organization NPI or Unique Minnesota Provider Identifier (UMPI), and the provider type on the first page of this agreement. This information will automatically populate to pages 2 and 3 after you enter it.
- An administrator, manager, director or other person authorized to sign must initial each page.
- Write the name and title of the person signing, and sign the last page of this agreement. This person must also be disclosed on the [Disclosure of Ownership and Control Interest of an Entity \(DHS-5259\) \(PDF\)](#).

Check if signing electronically:

- ☐ I am signing this form electronically. My name as typed in the signature field is my legally binding signature. I understand that my electronic signature has the same legal effect and can be enforced in the same way as a handwritten signature. (Minnesota Statutes, 325L.02(h), 325L.05 and 325L.08)

NAME OF PERSON SIGNING (TYPE OR PRINT)	TITLE	
SIGNATURE		DATE

Keep a copy of the Provider Agreement for your files and upload the original form, along with all other required documentation, using the online [Minnesota Provider Screening and Enrollment \(MPSE\) portal](#), or fax to the appropriate number as follows:

- Personal care provider organizations, fax to 651-431-7465.
- Home and community-based waiver providers, fax to 651-431-7493.
- All other provider types, fax to 651-431-7462.